

2026 IDSA/ATS GUIDELINE FOR COMMUNITY-ACQUIRED PNEUMONIA

Clinical Diagnosis, CURB-65 Triage & Site-of-Care Decisions

1. CLINICAL DIAGNOSTIC CRITERIA

Community-Acquired Pneumonia (CAP) requires radiographic demonstration of pulmonary infiltrate (consolidation on chest X-ray or CT) accompanied by acute clinical features including fever, cough, purulent sputum production, dyspnea, pleuritic chest pain, and focal crackles on lung auscultation.

2. CURB-65 RISK STRATIFICATION CRITERIA

Assign 1 point for each feature present:

- ? C: Confusion (abbreviated mental score ≤ 8 , or acute disorientation)
- ? U: Blood Urea Nitrogen > 19 mg/dL (or serum urea > 7 mmol/L)
- ? R: Respiratory rate ≥ 30 breaths per minute
- ? B: Blood pressure (Systolic < 90 mmHg or Diastolic ≤ 60 mmHg)
- ? 65: Age ≥ 65 years

SITE-OF-CARE RECOMMENDATIONS BASED ON CURB-65:

- ? Score 0 or 1: Low risk (30-day mortality $< 1.5\%$). Suitable for outpatient management.
- ? Score 2: Moderate risk (mortality 9.2%). Inpatient hospital admission or closely monitored outpatient therapy.
- ? Score 3 to 5: High risk (mortality 15% to 40%). Urgent hospital admission; scores 4 or 5 mandate intensive care (ICU) evaluation.

MICROBIOLOGICAL TESTING:

Blood cultures and sputum Gram stain/culture are recommended in hospitalized patients with severe CAP, or those empirically treated for MRSA or *Pseudomonas aeruginosa*.

TABLE 1: CURB-65 RISK STRATIFICATION

- Score 0 - 1: Outpatient management (Mortality $<1.5\%$)
- Score 2: Inpatient ward admission (Mortality $\sim 9\%$)
- Score 3: Inpatient admission (Mortality $\sim 17\%$)
- Score 4 - 5: ICU admission indicated (Mortality up to 40%)

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Empiric Antimicrobial Regimens: Outpatient vs Inpatient Ward

3. OUTPATIENT EMPIRIC ANTIMICROBIAL THERAPY

A. Patients Without Comorbidities or Risk Factors:

- ? Amoxicillin: 1000 mg orally three times daily for 5 days (preferred Class I).
- ? Doxycycline: 100 mg orally twice daily for 5 days.
- ? Macrolide (Azithromycin 500 mg day 1, then 250 mg daily) only in areas with pneumococcal macrolide resistance < 25%.

B. Patients With Comorbidities (Heart, Lung, Liver, Renal Disease, Diabetes):

- ? Combination Therapy (Preferred):
 - Amoxicillin-Clavulanate 875/125 mg orally twice daily (or 2000/125 mg bid) PLUS Azithromycin 500 mg day 1, then 250 mg daily.
- ? Monotherapy:
 - Respiratory Fluoroquinolone: Levofloxacin 750 mg orally once daily, or Moxifloxacin 400 mg once daily.

4. INPATIENT NON-SEVERE CAP ANTIMICROBIAL REGIMENS

Administer first antibiotic dose immediately in the emergency department.

PREFERRED COMBINATION THERAPY:

- ? Beta-Lactam + Macrolide:
 - Ceftriaxone 1 to 2 g IV once daily PLUS Azithromycin 500 mg IV or oral daily.
 - Alternative beta-lactams: Ampicillin-Sulbactam 1.5 to 3 g IV every 6 hours, or Cefotaxime 1 to 2 g IV every 8 hours.

PREFERRED MONOTHERAPY:

- ? Levofloxacin 750 mg IV or oral once daily, or Moxifloxacin 400 mg IV or oral once daily.

TABLE 2: EMPIRIC CAP DOSAGES

Outpatient Healthy: Amoxicillin 1 g tid or Doxycycline 100 mg bid
Outpatient Comorbid: Augmentin 875/125 bid + Azithromycin 500/250
Inpatient Non-Severe: Ceftriaxone 1-2 g IV qd + Azithromycin 500 mg
Respiratory Quinolone: Levofloxacin 750 mg qd (Alternative)

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Severe CAP, MRSA/Pseudomonas Protocols & Treatment Duration

5. SEVERE CAP & ICU REGIMENS

Severe CAP requires combination therapy:

? Ceftriaxone 2 g IV once daily PLUS Azithromycin 500 mg IV daily (or Levofloxacin 750 mg IV daily).

RISK FACTORS FOR MRSA & PSEUDOMONAS AERUGINOSA:

Prior respiratory isolation of the pathogen or recent hospitalization with IV antibiotic exposure in past 90 days.

? If MRSA suspected: Add Vancomycin 15 to 20 mg/kg IV every 8 to 12 hours (target serum trough 15 to 20 mcg/mL) OR Linezolid 600 mg IV every 12 hours.

? If Pseudomonas suspected: Prescribe antipseudomonal beta-lactam (Piperacillin-Tazobactam 4.5 g IV every 6 hours, or Cefepime 2 g IV every 8 hours, or Meropenem 1 g IV every 8 hours).

6. DURATION OF ANTIMICROBIAL THERAPY

? Minimum duration is 5 days.

? Discontinuation Criteria: Patient must be afebrile for ≥ 48 hours and exhibit no more than 1 sign of clinical instability:

- Heart rate ≤ 100 bpm
- Respiratory rate ≤ 24 breaths/min
- Systolic BP ≥ 90 mmHg
- Oxygen saturation $\geq 90\%$ on room air
- Normal mental status

Procalcitonin levels can assist in guiding early cessation of antibiotic therapy.